Everolimus + exemestane

Usual starting doses are everolimus 10 mg once daily and exemestane 25 mg once daily; use the doses on your prescription. Both tablets are taken every day continuously. There is no planned rest week. Review benefit and tolerability over time; there is no fixed adjuvant completion date.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Columns show sequence and general intervals, not equal periods or booked appointments. The team confirms who arranges each test, who reviews results and who authorises treatment or restart.

Check	Before starting	First eight weeks	Four-week reviews	Longer-term treatment	Hold / change / completion
Treatment	Confirm both prescriptions	Both medicines daily; no rest week	Continue daily unless held	Review benefit and tolerability	Confirm each medicine's plan separately
Blood counts / renal / liver	Baseline, including urea and urinary protein	Every four weeks or as indicated	Every four weeks or as indicated	Individualise additional renal/protein checks	Review abnormalities before restart
Glucose / lipids	Glucose, cholesterol and triglycerides	Lipids every four weeks; glucose as indicated	Continue metabolic monitoring	Adapt to diabetes, results and symptoms	Follow unresolved abnormalities
Mouth / lungs / infection	Review mouth, breathing and infection	Mouth-care prevention; report symptoms early	Review symptoms and daily function	Prompt assessment between visits	Confirm recovery and restart conditions
Bone / endocrine health	Review fracture risk and existing dual-energy X-ray absorptiometry (bone density) (DXA)	Exemestane symptom support	Follow the agreed bone-health plan	Repeat assessment according to risk	Endocrine and bone plan reviewed separately
Cancer response	Baseline disease assessment	New symptoms can bring review forward	Imaging according to consultant plan	Continue response and symptom review	Progression prompts a new treatment decision

THE WHOLE TREATMENT JOURNEY

Everolimus + exemestane: treatment and checks

Postmenopausal HR-positive / HER2-negative advanced breast cancer

Usual starting doses are everolimus 10 mg once daily and exemestane 25 mg once daily; use the doses on your prescription. Both tablets are taken every day continuously. There is no planned rest week. Review benefit and tolerability over time; there is no fixed adjuvant completion date.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First eight weeks	Four-week reviews	Longer-term treatment	Hold / change / completion
Treatment NCCP 00322 · SMPC	Confirm both prescriptions	Both medicines daily; no rest week	Continue daily unless held	Review benefit and tolerability	Confirm each medicine's plan separately
Blood counts / renal / liver NCCP 00322 · SMPC	Baseline, including urea and urinary protein	Every four weeks or as indicated	Every four weeks or as indicated	Individualise additional renal/protein checks	Review abnormalities before restart
Glucose / lipids NCCP 00322 · SMPC	Glucose, cholesterol and triglycerides	Lipids every four weeks; glucose as indicated	Continue metabolic monitoring	Adapt to diabetes, results and symptoms	Follow unresolved abnormalities
Mouth / lungs / infection NCCP 00322 · SMPC	Review mouth, breathing and infection	Mouth-care prevention; report symptoms early	Review symptoms and daily function	Prompt assessment between visits	Confirm recovery and restart conditions
Bone / endocrine health NCCP 00322 · SMPC	Review fracture risk and existing DXA	Exemestane symptom support	Follow the agreed bone-health plan	Repeat assessment according to risk	Endocrine and bone plan reviewed separately
Cancer response NCCP 00322 · SMPC	Baseline disease assessment	New symptoms can bring review forward	Imaging according to consultant plan	Continue response and symptom review	Progression prompts a new treatment decision

What each check is for

1 Before the first dose

Before starting

Sources: NCCP 00322 · SMPC

2 Early toxicity checks

First eight weeks

Sources: NCCP 00322 · SMPC

3 Benefit, tolerability and adherence

Longer-term treatment

Sources: NCCP 00322 · SMPC

4 Completion or treatment change

Hold / change / completion

Sources: NCCP 00322

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.21 · Sources checked 2026-10-25

Source reconciled; independent clinical, pharmacy and patient/PPI review pending